

India's Journey Towards Universal Health Coverage



For Prelims: Ayushman Bharat Vay Vandana , Non-communicable diseases , U-WIN platform , Tele-MANAS , Eat Right India , National Health Mission

For Mains: Universal Health Coverage (UHC) in India: Progress and Challenges, Ayushman Bharat as a Pillar of Welfare Governance, Public Health Financing and India's Health Expenditure Gap

Source: PIB

Why in News?

India's progress towards **Universal Health Coverage (UHC)** is in focus after the **National Statistical Office 2025 survey** highlighted increased use of public healthcare facilities, lower out-of-pocket expenditure, and improved access to affordable healthcare, driven by initiatives such as **Ayushman Bharat** and the **National Health Mission** .

Summary

- India has made significant progress towards Universal Health Coverage through Ayushman Bharat, Ayushman Arogya Mandirs, digital health platforms, expanded medical infrastructure, and targeted public health interventions, resulting in improved healthcare access, affordability, and health outcomes.
- Low public health expenditure, rising non-communicable diseases, regional disparities, governance gaps, and continued out-of-pocket expenditure highlight the need for higher investments, stronger regulation, digital integration, and a robust Right to Health framework.

What are the Achievements in India's Healthcare?

Universal Health Coverage & Insurance

- **AB-PMJAY:** Launched in 2018 under the [National Health Policy 2017](#), the [Ayushman Bharat Pradhan Mantri Jan Aarogya Yojana \(AB-PMJAY\)](#) provides free health insurance up to Rs 5 lakh/family per year to 12 crore socio-economically deprived families.
- **Scale and Reach:** Over **44.14 crore Ayushman Cards** have been created, funding 12.03 crore hospitalizations. The Ayushman App, available in 19 languages, simplifies access.
- **Senior Citizen Coverage:** The October 2024 launch of [Ayushman Bharat Vay Vandana](#) extended coverage to all citizens above 70 years.
 - By June 2026, 1.20 crore seniors were enrolled, availing treatments worth Rs 3,000 crore.
- **Case Study:** *Soni Khatoon* from Bihar, whose family was drowning in debt, received a completely free heart valve replacement surgery under AB-PMJAY in Lucknow.

Democratizing Primary Care

- **Ayushman Arogya Mandirs (AAMs):** Over **1.86 lakh facilities** (including Sub-Centres, PHCs, and AYUSH centres) have been upgraded to provide 12 free comprehensive services.
 - These centers have recorded a massive **540 crore cumulative footfall**.
- **Case Study:** The *Lalmati sub-centre in Assam* transformed into an AAM, introducing [Non-communicable diseases \(NCDs\)](#) screening and mental health support.
 - Led by Community Health Officers, the **area achieved zero maternal and child mortality from preventable causes since 2024**.

Pandemic Preparedness & Infrastructure

- **PM-ABHIM:** The [Pradhan Mantri Ayushman Bharat Health Infrastructure Mission](#) is building resilient infrastructure, including 744 integrated public health labs and 631 critical care hospital blocks in all districts with a population over 5 lakh.
- **COVID-19 Response:** India executed the **world's largest vaccination drive with over 220 crore doses**, scaled **ICU beds from 2,168 to 1.45 lakh**, and supplied nearly **300 million vaccine doses to 100 countries** under [Vaccine Maitri](#).

Maternal, Child Health & Disease Elimination

- **Safe Motherhood:** Schemes like [Pradhan Mantri Surakshit Matritva Abhiyan \(PMSMA\)](#) (7.47 crore pregnant women examined) and [Janani Shishu Suraksha Karyakram \(JSSK\)](#) ensure zero out-of-pocket costs for deliveries.
 - The active monitoring by [ASHA workers](#) ensures safe institutional births.
- **Immunization:** [Mission Indradhanush](#) vaccinated 5.46 crore previously unimmunized children.
 - As of April 2026, the "zero-dose" child population has dropped to just 0.06%.
- **Combating TB and Malaria:** The [Pradhan Mantri TB Mukht Bharat Abhiyaan](#) screened 20 crore people, while 3.78 lakh [Nik-shay Mitras](#) provided food baskets to 20 lakh patients. Malaria mortality has dropped by 78%, and mother-to-child HIV transmission has fallen by 74.5% since 2010.

Managing Non-Communicable Diseases (NCDs)

- **Mass Screenings:** [Ayushman Arogya Mandirs \(AAMs\)](#) have conducted staggering screening numbers: 41.5 crore for hypertension, 41.3 crore for diabetes, and over 60 crore combined screenings for oral, breast, and cervical cancers.
- **Dialysis & Cancer Care:** The [Pradhan Mantri National Dialysis Programme \(PMNDP\)](#) delivered 4

crore free hemodialysis sessions. Cancer care facilities have been approved in all 22 new AIIMS.

- **Lifestyle Interventions:** The **Eat Right India and Fit India Movement** tackle root causes of NCDs.
 - India also won the **2025 Bloomberg Philanthropies Award** for reducing tobacco use by 17.3% and implementing a successful **National Tobacco QuitLine**.

Digital Health & Last-Mile Delivery

- **ABDM & U-WIN:** Over 20.49 crore unique **Ayushman Bharat Health Accounts (ABHAs)** have been created. The **U-WIN platform** digitally tracks vaccinations for 11.87 crore children and 3.96 crore pregnant women.
- **Telemedicine & Drones:** **eSanjeevani** has delivered over 47 crore teleconsultations.
 - **Tele-MANAS** provides mental health support across 53 cells in 20 languages. Furthermore, the **ICMR's i-DRONE** delivered 22,000 medicines across 7,700 km of difficult terrain.
- **Artificial Intelligence:** AI is revolutionizing diagnostics via Clinical Decision Support Systems (CDSS), the **Cough Against TB tool** (detecting 12-16% additional hidden cases), and **MadhuNetrAI** for automated diabetic retinopathy screening.

Affordable Medicines, Diagnostics & Workforce Expansion

- **Cost Reduction:** Over 18,000 **Jan Aushadhi Kendras** (50-90% cheaper generic drugs) and 255 **AMRIT Pharmacies** (saved patients ₹8,400 crore) ensure affordability.
 - The *Free Diagnostics Initiative* covers up to 57 tests at district hospitals (**modeled successfully after Andhra Pradesh's NTR Vaidya Pariksha**).
- **Workforce & AYUSH:** Medical colleges have doubled, and 157 new nursing colleges are being established.
 - The Ministry of AYUSH has integrated traditional medicine across 13,093 facilities and introduced a dedicated **AYUSH Visa in July 2023 for global medical tourism**.

What are the Challenges in India's Healthcare?

- **Fiscal Deficits :** The **National Health Policy (NHP) 2017** target of raising public health spending to **2.5% of the GDP by 2025 remains unfulfilled** , severely limiting the structural expansion of public facilities.
 - Post-Covid-19, the Union government's health spending has experienced a noticeable contraction, dropping drastically from **0.37% of GDP in 2020-21 to a mere 0.29% of GDP in the 2025-26 fiscal year** .
 - The Union's share of health transfers to states for vital **Centrally Sponsored Schemes** has plummeted from **75.9% in 2014-15 to just 43% in 2024-25** .
 - This reduction weakens state-level fiscal capacity to maintain and operate frontline health infrastructure.
- **Low Per-Capita Public Investment:** India's per capita public health spending remains among the lowest globally.
 - India spends significantly less than **Bhutan (which spends 2.5 times more)** and **Sri Lanka (3 times more)** .
 - The disparity widens against other emerging markets; peer **BRICS nations spend 14 to 15 times more** per capita on public health, while **ASEAN** countries like Thailand and Malaysia outspend India by

10 times.

- **Siloed Institutional Operations:** Although NHP 2017 advocates for a holistic **“Health in All Policies” framework**, major health determinants such as sanitation, nutrition, clean water, and air pollution are handled by disparate ministries with independent budgets.
 - This institutional fragmentation **prevents the pooling of resources for cumulative outcomes**. It leaves public health systems to deal with preventable downstream diseases.
 - "Public health and sanitation; hospitals and dispensaries" currently sit under **Entry 6 of the State List (Seventh Schedule)**. This constitutional arrangement hampers the enforcement of uniform, legally binding **national standards**, leading to immense regional disparities in care quality.
- **NITI Aayog Revelations:** An evaluation study commissioned by **NITI Aayog** revealed that patients availing treatment in private hospitals under the **PMJAY** continue to incur **significant out-of-pocket expenditure (OOPE)**.
 - Despite PMJAY being heavily promoted as a completely cashless scheme, the study highlights that **65% of the beneficiaries still had to pay from their own pockets**, while a mere **35% enjoyed completely cashless hospitalization**, exposing major gaps in regulatory oversight and empanelment compliance.
- **Absence of Statutory Backing:** India lacks a centralized, legally enforceable **Right to Health Act** that would legally mandate adequate health provisioning, infrastructure access, and state accountability for its citizens.
- **Non-Communicable Disease Explosion:** NCDs (heart disease, diabetes, cancer) now account for **60% of all deaths** in India, driven by sedentary lifestyles and tobacco use, placing immense pressure on tertiary care and requiring constant, lifelong medical management.
- **Geographical and Physical Barriers:** Reaching remote, hilly, or underserved rural areas with advanced medical supplies (like vaccines and blood samples) and specialist doctors remains a logistical hurdle, historically resulting in "zero-dose" children and high maternal mortality.
- **Mental Health Stigma and Access:** Mental healthcare has historically been heavily stigmatized and under-resourced, with psychiatric care concentrated almost exclusively in major urban centers, leaving rural populations isolated.
- **Diagnostic Gaps in Traditional Systems:** Relying solely on conventional screening methods previously resulted in missed diagnoses for complex diseases like Tuberculosis and diabetic retinopathy in primary care settings lacking specialist doctors.

What Measures Can Strengthen Healthcare?

- **Mandating a Fiscal Correction Path:** Establish a legally backed fiscal roadmap to hit the public health expenditure target of 2.5% to 3% of GDP.
 - The Union government must reverse its post-pandemic expenditure decline and scale its contribution to meet the 1% of GDP mandate.
- **Scaling AI-Powered Diagnostics:** Expanding the deployment of AI tools like **MadhuNetrAI** and **Cough Against TB** across all rural AAMs to eliminate the need for physical specialists at the point of initial screening.
- **Expanding Drone Logistics:** Institutionalizing the **i-DRONE network nationwide to ensure that blood samples**, vaccines, and essential medicines can bypass difficult physical terrain seamlessly.
- **Sustaining the "Jan Andolan" (People's Movement) Approach:** Replicating the success of community-led initiatives like the *Nik-shay Mitras* for TB nutrition and the *Eat Right India* certifications to combat the rise of NCDs through community behavioural change.

- **Unifying Traditional and Modern Medicine:** Accelerating the rollout of the *AYUSH Grid* to digitally connect all traditional hospitals and labs, fully establishing India as the world's premier destination for holistic healing and medical tourism via the AYUSH Visa.
- **Deepening Digital Portability:** Ensuring 100% adoption of the 14-digit ABHA ID and the U-WIN platform across all private and public hospitals to create a truly flawless, paperless continuity of care.
- **Enacting a Right to Health Framework:** Legally pass a comprehensive *Right to Health Act* to establish health provisioning as a justiciable right, legally obligating the state to build resilient infrastructure and eliminate hidden costs.

Conclusion

India's sustained investments in healthcare infrastructure, digital health, preventive care, and financial protection through Ayushman Bharat have significantly advanced **Universal Health Coverage**, reducing healthcare inequalities and improving health outcomes. These efforts contribute directly to **SDG 3 (Good Health and Well-being)** and support the vision of **Viksit Bharat @ 2047** by building a healthier and more productive population.

Drishti Mains Question:

Universal Health Coverage is not merely a health objective but a socio-economic imperative. Discuss.

Frequently Asked Questions (FAQs)

1. What is Ayushman Bharat PM-JAY and why is it significant?

Ayushman Bharat PM-JAY provides health insurance coverage of ₹5 lakh per family annually to vulnerable households, reducing catastrophic health expenditure and advancing Universal Health Coverage.

2. What role do Ayushman Arogya Mandirs play in healthcare delivery?

Ayushman Arogya Mandirs strengthen primary healthcare by providing comprehensive services including NCD screening, maternal care, mental health support, and preventive healthcare at the grassroots level.

3. How is the Ayushman Bharat Digital Mission (ABDM) transforming healthcare?

ABDM enables digital health records through ABHA IDs, improves portability of healthcare services, enhances efficiency, and supports seamless continuity of care across facilities.

4. What are the major challenges facing India's healthcare system?

Key challenges include low public health expenditure, rising burden of non-communicable diseases, regional disparities, fiscal constraints, and persistent out-of-pocket expenditure despite insurance coverage.

[Watch Video on YouTube:

▶ <https://www.youtube.com/embed/YH4bdHYjbYE>]

UPSC Civil Services Examination, Previous Year Questions (PYQs)

Prelims

Q. Which of the following are the objectives of ‘National Nutrition Mission’? (2017)

1. To create awareness relating to malnutrition among pregnant women and lactating mothers.
2. To reduce the incidence of anaemia among young children, adolescent girls and women.
3. To promote the consumption of millets, coarse cereals and unpolished rice.
4. To promote the consumption of poultry eggs.

Select the correct answer using the code given below:

- (a) 1 and 2 only
- (b) 1, 2 and 3 only
- (c) 1, 2 and 4 only
- (d) 3 and 4 only

Ans: (a)

Mains

Q. “Besides being a moral imperative of a Welfare State, primary health structure is a necessary precondition for sustainable development.” Analyse. (2021)

Shigellosis Outbreak in Kerala



Source: [TH](#)

Recently, Kerala is witnessing a surge in **shigellosis (bacillary dysentery) cases**, with multiple districts affected, prompting health advisories on hygiene, water safety, and confinement of infected individuals.

- **About Shigellosis:** Shigellosis, also called **bacillary dysentery**, is a severe **bacterial diarrhoeal disease** marked by fever, abdominal cramps, and bloody or mucoid stools.
 - It is caused by Shigella, a **gram-negative bacterium first isolated by Kiyoshi Shiga in 1897** after a dysentery epidemic in Japan.
- **Transmission:** The disease spreads mainly through the faeco-oral route via **contaminated water, food, hands, infected surfaces, or direct person-to-person contact**.
 - It is extremely infectious, as ingestion of even 10–100 bacteria can cause infection and trigger outbreaks.
- **Risk Groups: Children under five face** the highest risk of severe illness and death, though no individual is completely immune. The disease can spread rapidly within households, schools and other communal settings if strict hygiene is not followed.
- **Prevention:** Key preventive measures include boiling drinking water, avoiding stale and raw food, washing hands with soap for 20 seconds, maintaining personal hygiene, isolating infected children at home, and avoiding food handling or public gatherings for at least two weeks after symptoms subside.
- **Concern:** While mild cases may recover with hydration, severe cases require medical care and antibiotics.
 - However, **multi-drug resistant Shigella** is a growing concern in India, making laboratory testing, judicious antibiotic use, food safety checks, clean water supply and strong surveillance essential.

Read More: [Shigella Bacteria Outbreak](#)

Nipah Virus

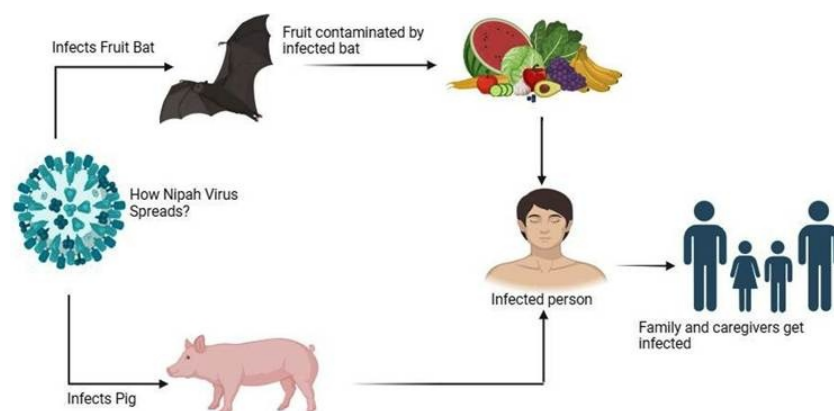


Source: [TH](#)

Recently, a **new Nipah virus case has been reported** in Kozhikode, **Kerala**, prompting immediate contact tracing, quarantine measures, and heightened surveillance to prevent human-to-

human transmission.

- **Nipah virus (NiV):** It is a **highly infectious zoonotic virus** , first identified in **1998-99 in Kampung Sungai Nipah, Malaysia** .
 - According to the **World Health Organization (WHO)** , it is transmitted from **animals to humans** , with **fruit bats (*Pteropodidae*)** as the natural reservoir and **pigs as intermediate hosts** . The virus belongs to the **Henipavirus genus** of the **Paramyxoviridae family** .
 - It has the **potential for human-to-human transmission** , making it a **serious public health threat that requires rapid surveillance and containment** .
- **Symptoms and Clinical Features:** Nipah infection usually begins with **influenza-like symptoms** such as **fever, muscle pain, sore throat, and respiratory distress** .
 - In severe cases, it can progress to **acute encephalitis** , leading to **convulsions, disorientation, coma** , and even death.
 - Importantly, **asymptomatic infections** are also reported, complicating containment efforts.
- **Diagnosis and Testing:** NiV is classified as a **Biosafety Level-4 (BSL-4) pathogen** , requiring testing in **high-security laboratories** .
 - Diagnosis is confirmed using **real-time polymerase chain reaction (RT-PCR)** , **ELISA** , **serum neutralisation tests** , **histopathology** , and **virus isolation** techniques.
- **Seasonal Risk:** Kerala faces peak Nipah spillover risk from **April to September** due to fruiting trees, increased bat activity, breeding season and viral shedding, making seasonal alerts crucial for early detection.
- **Long-term Concern:** Since fruit bats are the reservoir of **Nipah virus in Kerala**, recurrent spillover events may be difficult to prevent; hence, the **State's focus remains on surveillance, rapid diagnosis**, infection control and swift containment.
 - **Treatment and Prevention:** There is **no approved vaccine** for Nipah virus in humans or animals.
 - Treatment relies on **intensive supportive care and isolation** . India, particularly **Kerala** , has improved outcomes by using **monoclonal antibodies and antiviral drugs such as Remdesivir** , reducing mortality from **91% in 2018 to around 33% by 2023-25** .



Read More: [Nipah Virus](#)

Estonia Eyes Deeper Trade and Defence Ties with India



Source: TH

Estonian government officials and tech industry executives have expressed a strong desire to expand cooperation with India across critical sectors, particularly **defence, cybersecurity, trade, and artificial intelligence (AI)** .

- **India-Estonia Relations:** India first recognized Estonia in 1921 when it joined the **League of Nations**.
 - Following the dissolution of the **Soviet Union** , India re-recognized the Republic of Estonia in September 1991.
 - In 2024, the two nations held the inaugural **India-Estonia Cyber Dialogue** , acknowledging Estonia's status as a world leader in IT and digital governance, and India's massive scale in rolling out **Digital Public Infrastructure**.
 - Bilateral trade between India and Estonia stood at **139.3 million euros in goods** and **66.4 million euros in services** in 2025.
 - As of December 2025, **Indian FDI in Estonia amounted to 13.6 million euros** , while **Estonian FDI in India totalled 4.15 million euros** during the period 2000–2025.

Estonia

- **Geographical Location:** Estonia is a country located in **northeastern Europe** and is the northernmost of the **three Baltic states** (alongside Latvia and Lithuania).
 - It shares land borders with **Russia in the east** and **Latvia in the south** .
 - The country is bounded by the **Gulf of Finland in the north** , the **Baltic Sea in the west** , and **Lake Peipus in the east** . It also shares **maritime borders with Finland and Sweden** .
- **Islands:** Estonia's territory encompasses around 1,500 islands and islets. The two largest islands, **Saaremaa and Hiiumaa** , are located off the mainland's western coast.
- **Topography & Rivers:** The highest point in the country is **Suur Munamägi (318m)** . Its major rivers include the **Narva, Pärnu, Pedja, and Kasari** .
- **Basic Demographics:** The capital and largest city is **Tallinn** and it functions as a **parliamentary republic** .
- **Historical Timeline:** After being ruled by various powers including Denmark, Sweden, and the Russian Empire, Estonia saw its first period of independence in 1918. It was forcibly incorporated into the **USSR in 1940** .
 - It remained a Soviet constituent republic until **1991** , when it successfully declared its independence during the collapse of the Soviet Union.
- **Strategic Integration:** Following its transition to a parliamentary democracy and market economy, the country integrated with major Western institutions, becoming a full member of the **European Union** , the **Eurozone** , **North Atlantic Treaty Organization** , and the **Nordic-Baltic Eight (NB8)**.



Read more: [India-Estonia](#)